INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Weekly paclitaxel

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
28 days
Intervals differ by treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00226
Version 9

Paclitaxel on Days 1, 8, 15 and 22 of a 28-day cycle; a Days 1, 8 and 15 schedule may be selected by the consultant. Confirm your appointment plan.

Treatment stages

01 **Days 1, 8, 15 and 22**
Four weekly visits

02 **Days 1, 8 and 15 (if prescribed)**
Three visits + an interval

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

Understanding your treatment

Weekly paclitaxel · National Cancer Control Programme (NCCP) 00226

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. New chest pain, breathing difficulty, severe diarrhoea, inability to drink, reduced urine, uncontrolled bleeding or a rapidly worsening symptom needs prompt assessment.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your medicines and what they do

This weekly paclitaxel protocol is used in specified advanced breast, ovarian, small-cell lung and bladder-cancer settings. The aim is usually disease control and symptom relief. Evidence and alternatives differ between cancers; confirm the indication with your oncologist.

TAXANE CHEMOTHERAPY

Paclitaxel

Taxol (conventional paclitaxel)



Locks the cell division machinery

Cells use tiny structures called microtubules to separate their chromosomes when dividing. Paclitaxel stabilises these structures so they cannot rearrange normally, interrupting cell division. It can also affect nerves and other healthy tissues.

Microtubules form → Structures remain locked → Division is interrupted

What to watch for

Report tingling or numbness before the next weekly dose, especially if it affects walking or hand use. Tell the nurse immediately about flushing, chest or throat tightness, itching, dizziness or breathing changes during infusion.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Which cancer setting does this page cover?

The regimen number matters as well as the drug name.

Advanced breast cancer

The NCCP indication concerns metastatic breast cancer after unsuitable or unsuccessful anthracycline-based treatment. Tumour biology and previous treatments influence options. This page does not describe every early-breast-cancer paclitaxel regimen.

Ovarian, small-cell lung and bladder cancer

These are specific later-line settings in this shared protocol. Evidence is less robust than for the breast-cancer weekly comparison. Ask about current alternatives and the implications of use outside the licensed indication where applicable.

Paclitaxel formulations and combinations

Conventional paclitaxel and nab-paclitaxel have different formulations and administration requirements. This is a single-agent paclitaxel guide. If another anti-cancer medicine is added, obtain the matching combination guide.

What matters to you?

Discuss symptom control, time in hospital, hair loss, neuropathy risk and the activities you most want to preserve. Your priorities can change. Supportive care is available alongside active treatment.

Why this treatment may be chosen

Weekly paclitaxel can shrink or slow metastatic breast cancer. In a breast cancer study it produced more tumour responses than treatment given less often, but more severe nerve symptoms. Some trial participants also received trastuzumab, so the result does not predict every monotherapy outcome.

The treating team considers the cancer, previous treatment, health and your priorities. Trial averages do not predict an individual outcome.

SACTCHECK · CHAPTER 02

Your schedule, tests and appointments

Weekly paclitaxel · NCCP 00226

Your treatment plan

Treatment stages

01 **Days 1, 8, 15 and 22**
Four weekly visits

02 **Days 1, 8 and 15 (if prescribed)**
Three visits + an interval

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

FOUR WEEKLY VISITS

Days 1, 8, 15 and 22

D 1 | D 8 | D 15 | D 22

Four weekly infusion visits within each 28-day cycle. The team reviews safety before each planned dose.

THREE VISITS + AN INTERVAL

Days 1, 8 and 15 (if prescribed)

D 1 | D 8 | D 15 | Interval

Day 22 is omitted only if the three-visit schedule is prescribed. Follow your confirmed appointments and prescription.

The treating team confirms the indication, schedule and duration. Changes are recorded on your own prescription.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Milestones show order, not equal time intervals or booked appointments. The treating team requests tests, reviews results and confirms treatment. full blood count (FBC), renal and liver profile before each treatment, including each weekly dose. Review neuropathy, function, infusion reactions and symptoms before every administration. Default Days 1, 8, 15 and 22 every 28 days. The consultant may choose Days 1, 8 and 15 only; confirm the selected plan. Each dose needs its own review.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00226 v9 · official regimen · HPRA conventional paclitaxel product information; 09 January 2026

THE WHOLE TREATMENT JOURNEY

Weekly paclitaxel: treatment and checks

Shared advanced-cancer protocol

Paclitaxel on Days 1, 8, 15 and 22 of a 28-day cycle; a Days 1, 8 and 15 schedule may be selected by the consultant. Confirm your appointment plan.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Ongoing treatment	Dose change / pause	Next decision
Treatment plan NCCP 00226	Confirm indication, goal and current prescription	Continue the agreed weekly pattern while helpful and tolerated	Continue according to response, tolerance and prescribed course	Team review before restart or dose change	Agree completion or next treatment
Blood / kidney / liver NCCP 00226 · SMPC	Baseline FBC, kidney and liver tests	FBC, kidney and liver tests before each weekly treatment	FBC, kidney and liver tests before each weekly treatment	Recheck recovery and dose before restarting	Follow unresolved abnormalities
Symptoms / medicines NCCP 00226 · SMPC	Review symptoms, medicines and previous treatment	Review effects before each dose	Continue symptom and medicine reviews	Report problems; do not change or restart doses yourself	Agree recovery and next treatment plan
Heart / electrocardiogram (ECG) / echo NCCP 00226 · SMPC	Review cardiac symptoms and monitor observations during infusion. No fixed routine echo schedule is imposed for this paclitaxel monotherapy; significant conduction abnormalities require cardiac monitoring.	Review symptoms and indicated tests	Continue the risk- and treatment-specific plan	Urgent assessment for chest pain, fainting or breathlessness	Agree follow-up from exposure and results

Care / check	Before starting	Early treatment	Ongoing treatment	Dose change / pause	Next decision
Response / course NCCP 00226 · SMPC	Confirm cancer-specific treatment aim	Continue the agreed weekly pattern while helpful and tolerated	Review benefit, symptoms and the agreed scan plan	Reassess if treatment is held or symptoms change	Agree continuation, change or completion
Bone health / other care NCCP 00226 · SMPC	Review previous/ ongoing treatments and fracture risk	 No automatic chemotherapy -cycle dual-energy X-ray absorptiometry (bone density) (DXA)	Keep any separate endocrine/ fracture-risk plan	Review if clinical risks change	Continue separately indicated follow-up

What each check is for

1



Treatment and safety review

Before each planned treatment or four-week review for olaparib

FBC, renal and liver profile before each treatment, including each weekly dose. Review neuropathy, function, infusion reactions and symptoms before every administration.

Ask / act: The treating team confirms results and the plan; this map does not authorise a dose.

2

Benefit and continuation

At agreed clinical review points

Weekly paclitaxel can shrink or slow metastatic breast cancer. In a breast cancer study it produced more tumour responses than treatment given less often, but more severe nerve symptoms. Some trial participants also received trastuzumab, so the result does not predict every monotherapy outcome.

Ask / act: Discuss benefit, toxicity and priorities before continuing or changing treatment.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your practical treatment plan

Continue the agreed weekly pattern while helpful and tolerated

STEP 1

Weekly visits

Each planned infusion needs blood results, symptom review, preparation, premedication and observation. The one-hour infusion is not the total visit length.

STEP 2

Access and transport

Use a peripheral cannula or existing access selected with the team. A port is not automatic. Sedating antihistamines may affect driving; arrange transport as advised.

STEP 3

At home

Confirm whether Day 22 is prescribed and who to contact about missing results or appointments. Keep the medicine list and symptom record; report nerve symptoms early.

Before each weekly visit

Use the time between appointments to make review more informative.

Write down change from your baseline

Record tingling, numbness, pain, balance, bowel symptoms and fatigue. Tell the team about fever or illness when it occurs; the record is not a reason to wait.

Check medicines and transport

Know whether premedication is taken at home or given in hospital. Bring your medicine list and tell the team about a previous infusion reaction. Sedating antihistamines can affect driving.

Know the agreed schedule

Check the planned treatment days and blood-test arrangements. A three-visit and four-visit schedule must be distinguished in your own prescription. The number of visits may change after clinical review.

What the checks are for

Symptoms, blood results and the treatment aim answer different questions.

Blood counts

Neutrophils protect against infection, haemoglobin carries oxygen and platelets help clotting. A normal previous result or feeling well does not prove the current counts are safe. Fever or sudden illness needs immediate advice.

Kidney and liver function

Organ function can change drug exposure. Vomiting or diarrhoea can cause dehydration and worsen kidney function. Report reduced urine, poor intake, jaundice and new medicines. The clinical assessment combines results with symptoms.

Describe symptoms in practical terms

Tell the team about the worst symptoms since the last visit, even if improved today. Examples include extra stools above your usual pattern, whether mouth pain affects drinking, and whether numbness affects buttons or balance.

Assessing cancer control

The review plan may use scans, symptoms and ability to manage daily life. Stable disease can be worthwhile. Ask what benefit would justify continuing and what alternatives would be considered if treatment stops helping.

Symptoms and supportive care

Weekly paclitaxel · NCCP 00226

Days 1, 8, 15 and 22

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Four weekly infusion visits within each 28-day cycle. The team reviews safety before each planned dose.

Treatment or check	Timing in this view
Paclitaxel Day 1	 **Day 1** intravenous (IV)
Paclitaxel Day 8	 **Day 8** IV
Paclitaxel Day 15	 **Day 15** IV
Paclitaxel Day 22 · if prescribed	 **Day 22** IV
Nerves / muscle aches	 **Days 1–28** May persist or accumulate
Blood counts / energy	 **Days 1–28** Review before each dose; report illness any day

Weekly visits: Each planned infusion needs its own symptom and blood-test review.

Nerves and aches: Aches can follow treatment; numbness, tingling and fatigue may persist or accumulate. Describe changes before the next dose.

Day 22: The fourth visit applies only to the four-visit plan. Some patients are prescribed Days 1, 8 and 15; follow your confirmed schedule.

Blood counts: A gap between visits does not exclude infection or another treatment complication. Fever or sudden illness needs immediate contact.

> **Why these bands stay broad:** Bars show planned treatment and possible symptom patterns, not measured risk. Side effects can overlap and persist. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00226 v9 · official regimen ↗ · eviQ: patient information (supportive explanation; NCCP schedule takes priority) ↗

> **There are no guaranteed "safe days".** Act on fever, sudden illness or a new serious symptom whenever it occurs. Do not wait for the next appointment.

Days 1, 8 and 15 (if prescribed)

Day 1 → Day 28: a general pattern

Day 22 is omitted only if the three-visit schedule is prescribed. Follow your confirmed appointments and prescription.

Treatment or check	Timing in this view
Paclitaxel Day 1	 **Day 1** IV
Paclitaxel Day 8	 **Day 8** IV
Paclitaxel Day 15	 **Day 15** IV
Nerves / muscle aches	 **Days 1–28** May persist or accumulate
Blood counts / energy	 **Days 1–28** Review before each dose; report illness any day

Weekly visits: Each planned infusion needs its own symptom and blood-test review.

Day 22: The fourth visit applies only to the four-visit plan. Some patients are prescribed Days 1, 8 and 15; follow your confirmed schedule.

Nerves and aches: Aches can follow treatment; numbness, tingling and fatigue may persist or accumulate. Describe changes before the next dose.

Blood counts: A gap between visits does not exclude infection or another treatment complication. Fever or sudden illness needs immediate contact.

> **Why these bands stay broad:** Bars show planned treatment and possible symptom patterns, not measured risk. Side effects can overlap and persist. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00226 v9 · official regimen ↗ · eviQ: patient information (supportive explanation; NCCP schedule takes priority) ↗

Between-treatment notes

Day 1: First visit of this cycle

Review symptoms, blood results and premedication before the infusion. Tell the nurse straight away about itching, flushing, throat or chest tightness, back pain, dizziness or breathing change. Check transport if premedication makes you drowsy and confirm the next weekly visit.

Day 2–7: Between weekly doses

Aches, fatigue, nausea or bowel symptoms may occur. Tingling may be mild at first but matters before the next dose. Record what symptoms stop you doing. Contact the team for new or worsening numbness, balance trouble, marked weakness or uncontrolled symptoms. Fever or sudden illness needs immediate contact.

Day 8: Second weekly review

This dose requires its own symptom and blood-test review. Describe the worst symptoms since the previous visit, not only how you feel today. Report sensory changes before the infusion and confirm your premedication instructions.

Day 9–14: Keep tracking function

Repeated treatment can increase nerve symptoms and fatigue. Check practical tasks such as buttons, writing and walking. Use the symptom passport to show changes. Seek early advice rather than waiting until numbness is severe.

Day 15: Third weekly review

The team decides whether treatment can proceed using the current assessment. Confirm whether Day 22 is planned for you; do not infer this from a different person’s calendar.

Day 16–21: After the third visit

Symptoms may settle or continue. An interval between appointments does not rule out infection or a treatment complication. Use the urgent contact plan if unwell and bring your record to the next review.

Day 22: Day 22: follow the prescribed schedule

A fourth dose is part of the four-visit plan. In the consultant-selected three-visit plan there is no Day 22 infusion. The calendar here is educational. Use your confirmed appointment and prescription; call if the schedule is unclear.

Day 23–28: Complete the cycle record

Record symptoms and functional changes, including those that improve before the next appointment. Treatment continuation depends on benefit, toxicity and your priorities. Check the next cycle date and review plan. Do not treat a gap in visits as a guarantee of recovery.

Support around the medicines

1 Premedication for infusion reactions

Steroid and antihistamine premedication reduce the chance or severity of reactions. The team may adjust premedication after tolerated doses. Do not omit or change it yourself; tell the nurse about missed doses.

2 Aches, nausea and bowel support

Agree a pain-relief and anti-sickness plan. Ask before taking new medicines. Uncontrolled pain or poor intake needs review. Some supportive medicines cause constipation or drowsiness.

3 Nerve symptoms and fall prevention

Use secure footwear and take care with balance, hot surfaces and sharp objects if sensation is reduced. Tell the team early about functional changes; persistent neuropathy may influence dosing.

4 Hair, nails and skin

Ask about hair-loss support and whether scalp cooling is appropriate and available. Protect sensitive nails and skin. Report painful nail changes, spreading rash, blistering or infection.

5 Review the full medicine plan

First-dose NCCP suggested premedication: dexamethasone 8 mg IV, chlorphenamine 10 mg IV and famotidine 20 mg IV 30 minutes before paclitaxel. From Day 8, dexamethasone may be omitted in the source suggested table; add or modify for increased risk or previous reaction under consultant guidance. Famotidine omission is also a consultant/local-policy decision requiring close observation. The conventional paclitaxel product information has a different generic premedication regimen. Reconcile the exact product, weekly NCCP pathway and local policy; never self-omit premedication. Low emetic risk; keep the antiemetic plan separate from reaction premedication. Consider analgesia for myalgia/arthralgia and early action on neuropathy.

What to watch for

Some effects appear early; others build over repeated doses.

Neuropathy and lasting effects

Paclitaxel can affect sensation and sometimes strength. Report numbness, burning pain, weakness or difficulty with everyday tasks before the next dose. Symptoms may take time to improve and can persist after treatment.

Aches and fatigue

Muscle or joint pain can occur after treatment. Use the agreed analgesia plan and describe how pain affects sleep and function. New extreme weakness, fainting or breathlessness should be assessed rather than attributed automatically to treatment.

Hair, skin and nails

Hair loss is common with paclitaxel; it may include body hair. Nail changes and skin symptoms can occur. Ask about practical support and report painful, infected or rapidly worsening changes.

Mouth, bowels and infection

Gentle mouth care and early reporting help when soreness affects eating. Report persistent diarrhoea or vomiting. Low blood counts can make infection dangerous; a normal temperature does not make sudden illness safe.



Infusion and breathing symptoms

Reactions can occur during infusion despite premedication. Tell the nurse immediately. New cough or breathlessness after leaving also needs assessment; the cause may be infection, a clot, treatment or cancer.

Living with treatment

Plan around the activities and people that matter to you.

Food, drink and practical help

Use foods and drinks you can manage. Tell the team about weight loss, poor intake or difficulty shopping and cooking; ask for dietitian or practical support. A fixed daily fluid target is not appropriate for everyone, particularly with heart or kidney problems.

Activity, rest and driving

Balance rest with manageable activity. Record what fatigue stops you doing. Do not drive when dizzy, unwell or affected by sedating medicines. Ask about work adjustments and transport for treatment days.



Medicines, vaccines and supplements

Bring a complete medicine list, including over-the-counter products and supplements. Ask the oncology pharmacist before changes. Discuss vaccination timing. Do not stop prescribed long-term medicines on your own.

Fertility, contraception and breastfeeding

Discuss fertility preservation before treatment when relevant. Ask for a written contraception and breastfeeding plan covering each medicine and the period after treatment. Tell the team promptly about a possible pregnancy.

Support for you and your family

Ask about psycho-oncology, social work, counselling and symptom support. Take information in manageable amounts. A family member or trusted person can help record questions if you wish. Discuss how treatment fits your priorities.

Useful treatment terms

Short explanations for the clinic conversation.

Taxane and microtubule

A taxane is a chemotherapy drug family. Microtubules are small cell structures involved in division and other functions. Effects on these structures help explain both anti-cancer activity and nerve toxicity.

Premedication and neuropathy

Premedication is medicine given before an infusion to reduce reactions. Neuropathy means nerve dysfunction. These are separate issues and both need review during weekly therapy.

Benefit, course and continuing checks

Heart checks and the full course

Review cardiac symptoms and monitor observations during infusion. No fixed routine echo schedule is imposed for this paclitaxel monotherapy; significant conduction abnormalities require cardiac monitoring. Default Days 1, 8, 15 and 22 every 28 days. The consultant may choose Days 1, 8 and 15 only; confirm the selected plan. Each dose needs its own review.

Questions worth asking

Does having more side effects mean treatment is working?

No. Side effects do not reliably show whether the cancer is responding. Report them so they can be treated. Your team assesses benefit using the tests and review plan appropriate to your cancer.

Why might a dose be delayed or reduced?

Your team may allow recovery, investigate a symptom or reduce the chance of lasting harm. Ask which drug changes, why, and when review is planned. A delay does not itself mean treatment has failed.

What should I say when calling?

Give the regimen name, last treatment date, symptoms and when they started. Explain whether you can eat, drink, pass urine and manage usual activities. Give your temperature if available, but do not delay calling to collect details.

What belongs in the consent conversation?

The individual aim and likely benefit, uncertainty, reasonable alternatives including not proceeding, common and serious risks, lasting effects, fertility and practical concerns. You can ask for an interpreter, more time or another explanation and can reconsider your decision.

How many cycles should I use this passport for?

The passport provides reusable recording pages, not a prescribed course length. Your oncology team sets the duration and can provide extra pages.

Can I use this page to decide if blood results are safe?

No. Treatment decisions require the clinical protocol, symptoms, previous toxicity, organ function and the prescriber's review. The patient page explains the checks and does not authorise treatment.

> **Try explaining the plan back:** Can you explain which medicines you are receiving, the schedule, the contact plan and the symptoms that need prompt help?

Paclitaxel

- Low blood counts / infection
- Sensory or motor neuropathy, potentially persistent
- Hypersensitivity / infusion reaction
- Hair loss and nail / skin changes
- Muscle and joint aches
- Fatigue and mouth / bowel symptoms
- Rare serious lung or cardiac effects

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Weekly paclitaxel · reviewed supportive care

Active medicines: Paclitaxel

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally	On treatment day Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed

Medicine	Dose / route	When
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days: duration to be confirmed Assess contraindications, renal function and interactions

Additional phase-specific support

- First paclitaxel dose: dexamethasone 8 mg intravenous (IV), chlorphenamine 10 mg IV and famotidine 20 mg IV, each 30 minutes before treatment (NCCP suggested table).
- From Day 8 onward: chlorphenamine 10 mg IV and famotidine 20 mg IV 30 minutes before treatment. Routine dexamethasone may be omitted after the first dose; prior reaction or increased risk requires consultant review.
- Famotidine may be omitted under the consultant/local plan if appropriate. A prior reaction requires a separate rechallenge assessment; do not reuse the standard draft automatically.
- First-dose NCCP suggested premedication: dexamethasone 8 mg IV, chlorphenamine 10 mg IV and famotidine 20 mg IV 30 minutes before paclitaxel.
- From Day 8, dexamethasone may be omitted in the source suggested table; add or modify for increased risk or previous reaction under consultant guidance. Famotidine omission is also a consultant/local-policy decision requiring close observation.
- The conventional paclitaxel product information has a different generic premedication regimen. Reconcile the exact product, weekly NCCP pathway and local policy; never self-omit premedication.
- Low emetic risk; keep the antiemetic plan separate from reaction premedication. Consider analgesia for myalgia/arthralgia and early action on neuropathy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00226 · VERSION 9

Weekly paclitaxel

Paclitaxel on Days 1, 8, 15 and 22 of a 28-day cycle; a Days 1, 8 and 15 schedule may be selected by the consultant. Confirm your appointment plan.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Hands / feet

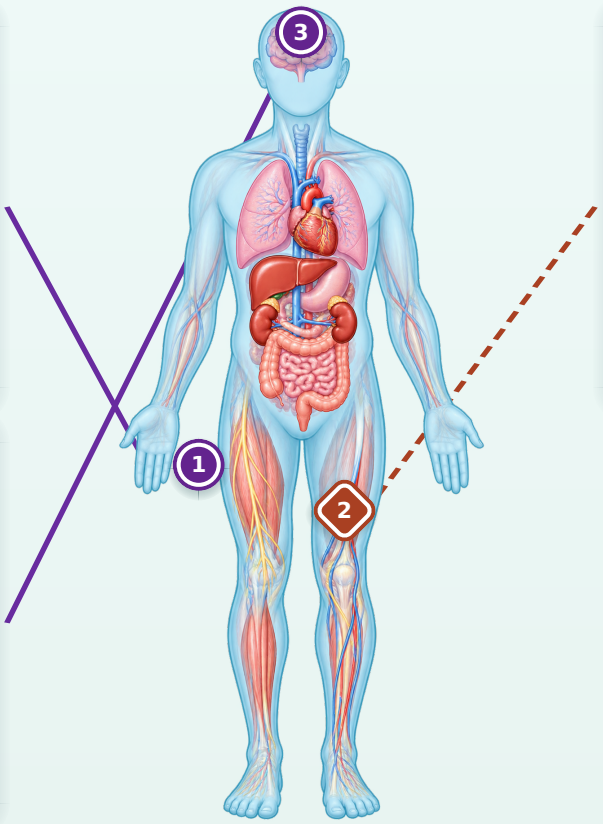
Paclitaxel

Report numbness or tingling before the next dose.

3 Hair / scalp

Chemotherapy phase

Hair loss is common. Ask about scalp cooling before treatment where suitable.



2 Blood / infection

Chemotherapy phase

Fever, shivers or suddenly unwell: call immediately.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Selected medicine-specific effects; the full risk discussion includes hair loss, fertility and other individual risks.

> Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

SACTCHECK · CHAPTER 04

Sources and further information

Weekly paclitaxel · National Cancer Control Programme (NCCP) 00226

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00226 v9 · official regimen**
   https://healthservice.hse.ie/documents/6562/226_V9_Paclitaxel_80.pdf
2. **HPRA conventional paclitaxel product information; 09 January 2026**
   https://assets.hpra.ie/products/Human/27560/Licence_PA2315-115-001_09012026104101.pdf
3. **CALGB 9840: weekly versus three-weekly paclitaxel (2008)**
   https://pubmed.ncbi.nlm.nih.gov/18375893/
4. **eviQ: patient information (supportive explanation; NCCP schedule takes priority)** ↗
   https://www.eviq.org.au/medical-oncology/breast/metastatic/42-breast-metastatic-paclitaxel-weekly/patient-information
5. **HSE: chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
6. **NCI: nausea and vomiting**
   https://www.cancer.gov/about-cancer/treatment/side-effects/nausea-vomiting
7. **NCCP antiemetic guidance; reconcile with regimen and individual prescription**
   https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_systemic anticancer therapy (SACT)_Induced_Nausea__pWgDCrm.pdf